



College of Health Science

School of Public Health

**Health Service Management Course
Group Assignment**

Primary Health Care

<u>NAME</u>	<u>Id.no.</u>
1. Eskedar Tsegaye	UGR/9290/13
2. Getayawkal Tesfaye	UGR/5538/13
3. Hanan Essa	UGR/5537/13
4. Haymanot Getu	UGR/2711/13
5. Hermela Desalegn	UGR/2737/13
6. Hermela Getahun	UGR/2057/13
7. Melat Mamo	UGR/5165/13
8. Nuhamin G/Silassie	UGR/8619/13
9. Rediet Kassaye	UGR/7022/13
10. Selamawit Mulugeta	UGR/2728/13
11. Tigist Girmay	UGR/7410/13

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Introduction to primary health care

Primary Health Care (PHC) refers to the essential health care services based on practical, scientifically sound, socially acceptable and universally accessible to individuals and families in the community through their full participation at an affordable cost to maintain every stage of their development in the spirit of self reliance and self determination.

Important terms in the definition

- Primary: First in order of its importance
- Essential : basic and indispensable
- Practical: Applicable
- Scientifically sound : scientifically explainable and acceptable
- Socially acceptable methods : acceptable by the community
- Universally accessible: To bring health care as close as possible to where people live and work
- Self reliance and self determination- Being independent understanding your own needs and trying to minimize problems.

According to World Health Organization (WHO), 'Primary Health Care is a basic health care and is a whole of society approach to healthy well-being, focused on needs and priorities of individuals, families and communities.

It acquaints all necessary elements at the community level to promote the overall health of the population that includes health promotion ,disease prevention, treatment, rehabilitation and palliative care. It is initiated on the notion that everyone in the country should have access to at least the basic and essential health services necessary for maintaining good health and receiving proper care.

Primary health care service is developed to attain the **objective** of

- Enhancing comprehensive services that determine the healthy growth and development of children
- Provision of effective and timely access to primary health care services
- Increasing community participation with the government with the joint aim of boosting the health of their community
- Advocating for development and implementation of healthy policy in all levels of government
- Implementing accountability standards within health care practice
- Promoting the community delight with the primary health care system

Historical development of primary health care

The World Health organization (WHO), which was established in **1948**, has had an objective of "the attainment by all people of the highest possible level of health." However due to political and socio-economic factors the various health care approaches implemented in different countries between **1948 and 1978** did not enable WHO to meet the stated objective. It failed to impact the health status of the population;unable to meet the demands of those most in need of health services. These situations called for revaluation.

The concept of Primary Health Care (PHC) was derived in the **1960s** as a response to the shortcomings of the health systems that developing countries practiced which were urban-focused, centralized, and primarily treatment-based, making them insufficient for meeting the actual needs of the population. It was amended during the international conference on primary health care held in Alma Ata, Kazakhsatnin **1978**, that held an interest on shifting of the health care services toward rural based, decentralized and preventive care approach.

This conference adopted “**The Alma Ata declaration**” that influenced global health policy and practice, and laid an immense ground for the adoption and development of primary health care as a central component of global health system.

It defined primary health-care as “essential health care based on practical, scientifically sound, and socially acceptable methods and universally accessible to individuals and families in the community through their full participation at an affordable cost to maintain at every stage of their development in the spirit of self-reliance and self-determination.”

It stressed on the significance of community participation, health promotion, and disease prevention in addition to curative services. And also acknowledged the right to health which emphasized the need for a comprehensive approach to healthcare that addressed not just physical health, but also mental, social, and spiritual well-being

The (WHO)’s “Health for all by the year 2000” initiative of **1977** although not being achieved, It raised an awareness for the need of accessible and affordable health for all, declared that primary health care strategy is the key to meet the goal. And this led to the establishment of primary health care in **1978’s**.

In **2008**, the WHO launched the “Primary health-care: now more than ever” report which highlighted several reforms necessary to make primary health-care effective.

In **2018** conference, a held in **Astana, kazakhsatn** ratifies the Astana Declaration which traced the progress of primary health-care since the Alma Ata declaration and established ways of strengthening primary health-care to achieve universal health coverage.

Philosophy and Principles of PHC

PHC (Primary Health Care) Philosophy is the **underlying belief system or vision** that guides PHC, while PHC Principles refer to the **core elements or guidelines** that define how PHC is implemented.

The key difference between the two lies in their **level of abstraction and purpose**. PHC Philosophy is more **ideological and value-based**, focusing on **why** PHC matters such as the belief that health is a basic human right. In contrast, PHC Principles are more **practical and programmatic**, outlining **what** PHC must include and **how** it should be delivered, like ensuring equity and community participation. Essentially, the philosophy sets the **vision**, while the principles provide the **blueprint** to bring that vision to life.

Both PHC Philosophy and PHC Principles deal with the **same core components** of primary health care things like **equity, community involvement, prevention, appropriate technology, and inter-sectoral collaboration**, but they approach them from different angles.

The PHC Philosophy

Equity and justice – Equitable distribution of services, resources and health care. – Health service should be accessible and affordable.

Individual and community self-reliance – Personal responsibility for their own and their family's health.

Inter-relationship of health and development – Health makes a fundamental contribution to a country's economy.

The PHC Principles

I. Equity:

The **philosophy** of equity in Primary Health Care (PHC) is rooted in the belief that health is a fundamental human right, and that everyone regardless of financial status, geographic location, or background deserves equal access to healthcare. This vision is supported by global declarations like the Declaration of Astana, which affirms the right of every human being to enjoy the highest attainable standard of health without discrimination. From a philosophical standpoint, equity is about promoting social justice by narrowing the gap between the “have's and the have not's,” and ensuring that health systems empower the community and protect the vulnerable. On the other hand, the **principle** of equity in PHC is the practical application of this belief. It emphasizes that all PHC services must be physically, socially, and financially accessible to everyone according to their needs. It focuses on removing barriers that prevent people especially the most disadvantaged from receiving care, and ensuring a fair distribution of health resources. This principle is reinforced in global goals like Sustainable Development Goal (SDG) 10, which targets the reduction of inequality. In action, equity means PHC systems must prioritize the most needy and vulnerable members of the community to achieve true universal health coverage.

II. Community participation:

The **philosophy** of community participation in Primary Health Care (PHC) is based on the belief that health services should be owned and driven by the community itself, promoting a sense of responsibility, empowerment, and sustainability. In Ethiopia, this is reflected in PHC's strong emphasis on community ownership, where people are seen not just as beneficiaries but as active partners in improving their own health. This belief values the community's potential, culture, and unique understanding of their health needs. Translating this belief into action, the **principle** of community participation guides the practical involvement of communities in every step of PHC from identifying and prioritizing problems to planning, implementing, monitoring, and evaluating health programs. Communities play a key role by contributing labor, sharing costs, engaging in decision-making, and managing services, which in turn strengthens coverage and ensures the services actually address the real problems of the society. This principle is also supported by SDG 16, which promotes inclusive and participatory health systems. For community participation to be meaningful, it must be encouraged through advocacy, belief in people's capabilities, respect for local culture, and building effective community structures.

III. Inter-sectoral collaboration:

The **philosophy** of inter-sectoral collaboration in Primary Health Care (PHC) is based on the understanding that health is not shaped by the health sector alone, but by a wide range of social, economic, and environmental factors. Ethiopia embraces this belief by recognizing that true health improvement requires collaboration across sectors, especially among education, agriculture, water, and sanitation. This philosophy reflects the idea that health is interconnected with how people live, learn, work, and access basic services. Translating this belief into practice, the **principle** of inter-sectoral collaboration calls for active partnerships between the health sector and other sectors like education, agriculture, housing, environment, water, and waste management. A narrow, health-only approach limits the effectiveness of PHC, while collaborative efforts save resources, address community needs more holistically, and improve overall health outcomes. This principle is also supported by SDG 17, which promotes partnerships and cooperation across sectors to achieve sustainable development. To make intersectoral collaboration successful, it's essential to create strong links, shared goals, and coordinated efforts between all involved sectors.

IV. Appropriate technology

The **philosophy** of appropriate technology in PHC is built on the belief that healthcare tools and methods should be low-cost, effective, culturally acceptable, and suitable for local use, especially in resource-limited settings. Ethiopia's PHC approach reflects this philosophy through the use of tools like family health cards, growth monitoring charts, clean delivery kits, and mobile health applications, which are simple yet impactful, and designed to fit the needs and realities of the communities they serve. This belief emphasizes that technology doesn't need to be expensive or high-tech to be valuable. It must be relevant, safe, and empowering. Translating this into action, the **principle** of appropriate technology ensures that the technologies integrated into PHC are scientifically sound, effective, socially acceptable, affordable, and environmentally safe. These tools should be locally sustainable, adaptable to the community's context, and undergo continuous evaluation to maintain quality and relevance. Appropriate technology improves efficiency and accessibility, and supports the long-term success of PHC programs, especially in underserved areas.

V. Emphasis on health promotion and prevention

The **philosophy** of health promotion and disease prevention in PHC is rooted in the belief that maintaining health is more effective and sustainable than simply treating illness. Ethiopia's PHC model reflects this by shifting the focus away from curative care alone and instead prioritizing preventive and promotive services through programs like immunization, maternal care, nutrition, and communicable disease control, mainly delivered by the Health Extension Program (HEP). This philosophy sees individuals and communities as active participants in managing their health, not just passive patients. Putting this belief into practice, the **principle** of emphasis on health promotion and prevention ensures that PHC services include empowering individuals and communities through awareness creation, behavior change, and lifestyle modification. Preventive measures in PHC aim to reduce the incidence of diseases, halt outbreaks, and address root causes at the individual level, while promotive strategies strengthen people's ability to stay healthy. In line with the

Declaration of Astana, PHC must also provide curative, rehabilitative, and palliative care to ensure a complete approach to health. These services not only prevent disease progression and disability, but also work toward improving quality of life for all individuals as needed.

Approaches to Primary Health Care (PHC)

Primary Health Care (PHC) is the cornerstone of health systems worldwide, aiming to provide accessible, equitable, and affordable health services to all individuals and communities. PHC is implemented through two main approaches:

- Comprehensive PHC (cPHC) and
- Selective PHC (sPHC)

Each approach has distinct characteristics, advantages, and disadvantages, tailored to different contexts and priorities.

1. Selective Primary Health Care (sPHC)

- Selective PHC is a targeted, low-cost strategy designed to address a few specific diseases or health issues that have a significant impact on mortality and morbidity.

Key Features of sPHC

- Focuses on treating and preventing a limited number of diseases.
- Prioritizes interventions based on criteria such as prevalence, severity, mortality risk, and feasibility of control.
- Measures success in terms of disease reduction and mortality rates.

Example: The *GOBI-FF* strategy:

- G: Growth monitoring
- O: Oral rehydration therapy
- B: Breastfeeding promotion
- I: Immunization
- F: Female education
- F: Family planning

Advantages of sPHC

1. Ease of Planning and Implementation: Limited scope makes it simpler to execute.
2. Faster Results: Focused interventions yield measurable outcomes quickly.
3. Resource Efficiency: Requires fewer financial and human resources.
4. High Satisfaction: Immediate impact on specific health issues increases community satisfaction.

Disadvantages of sPHC

1. Limited Scope: Does not address broader health issues or determinants of health.
2. Disease-Oriented Approach: Focuses on treating individuals rather than promoting overall community health.
3. Top-Down Decision Making: Community priorities and involvement are often overlooked.
4. Lack of Multi-Sectoral Collaboration: Fails to recognize the contributions of other sectors like education, sanitation, and agriculture.
5. Dependency: Encourages reliance on health services rather than empowering communities to manage their own health.

2. Comprehensive Primary Health Care (cPHC)

Comprehensive PHC is a holistic approach that addresses the broader determinants of health and aims to achieve equity and community empowerment.

Key Features of cPHC

- Defines health in a holistic sense, encompassing physical, mental, and social well-being.
- Emphasizes equity and accessibility for all individuals.
- Promotes multi-sectoral collaboration to address health determinants.
- Encourages active community participation in planning and implementation.

Advantages of cPHC

1. Holistic Approach: Addresses all aspects of health, including prevention, promotion, and treatment.
2. Community Empowerment: Involves communities in decision-making, fostering self-reliance.
3. Equity Promotion: Ensures health services are accessible to marginalized and vulnerable populations.
4. Multi-Sectoral Collaboration: Engages various sectors to tackle the root causes of health issues.
5. Addresses Community Priorities Tailor interventions to meet the specific needs of the community.

Disadvantages of cPHC

1. High Initial Costs: Requires significant investment in infrastructure and planning.
2. Complex Process: Involves multiple stakeholders and sectors, making implementation challenging.
3. Gradual Results: Long-term benefits take time to materialize.
4. Resource Intensive: Demands sustained financial and human resources.

Comparison of sPHC and cPHC

Aspect	Selective PHC (sPHC)	Comprehensive PHC (cPHC)
Focus	Disease-specific interventions	Holistic health and well-being
Resource Requirement	Low cost	High initial investment
Community Involvement	Limited	Active participation and empowerment
Decision Making	Top-down	Bottom-up, community-driven
Scope	Narrow, disease-oriented	Broad, addressing all health aspects
Results	Quick, measurable outcomes	Long-term, sustainable benefits

- Both Comprehensive PHC and Selective PHC have special advantages and disadvantages. While comprehensive PHC is best suited for attaining long-term, sustainable health improvements and equity, selective PHC is appropriate for handling urgent medical emergencies with limited resources.

The particular context, available resources, and national or local priorities all influence the method that is chosen. A strong health system that promotes long-term health and well-being while meeting urgent demands can be achieved through the balanced integration of both strategies.

Primary Health Care (PHC) Components

Alma-Ata Declaration (1978) – Original 8 Components

The International Conference on Primary Health Care in Alma-Ata (1978) defined eight essential components of PHC:

1. Health education
2. Nutrition promotion
3. Safe water supply and sanitation
4. Maternal and child health care (including family planning)
5. Immunization against major infectious diseases
6. Prevention and control of endemic diseases
7. Appropriate treatment of common diseases and injuries
8. Provision of essential drugs

These components were the foundation of the "Health for All" strategy.

Later Additions & Expansions (1990s–2020s)

Over time, the scope of PHC expanded in many countries and in WHO recommendations to include new priority areas due to changing global health needs:

- Oral health care
- Mental health care
- Rehabilitation and palliative care
- Care for non-communicable diseases (diabetes, hypertension, etc.)
- Environmental health, accident prevention, and elderly care

These additions reflected the growing burden of chronic diseases, aging populations, and the need for comprehensive care beyond infectious diseases.

WHO's Current PHC Framework (2025)

WHO now defines PHC as a **whole-of-society approach** with three broad components:

1. **Integrated health services across the life-course** – covering health promotion, prevention, treatment, rehabilitation, and palliative care.
2. **Addressing broader determinants of health** – through multisectoral policy and action (education, environment, economy, housing, etc.).
3. **Empowering individuals, families, and communities** – to take charge of their own health and participate in health decision-making.

This framework goes beyond listing services—it emphasizes equity, community empowerment, and intersectoral collaboration.

Primary Health Care Strategies

While the principles of Primary Health Care (PHC) have been recognized globally for decades, effective implementation remains a significant challenge. The COVID-19 pandemic exposed the fragility of health systems lacking a strong PHC foundation and created a renewed urgency for coordinated global action. In response, the World Health Organization (WHO) developed a comprehensive **three-pillar strategy**:

- 1) Providing tailored implementation support to Member States
- 2) Fostering evidence-based innovation with a sharp focus on equity
- 3) Promoting policy leadership and strategic partnerships

1. Providing tailored implementation support to Member States

The first pillar emphasizes country-specific support acknowledging that a "one-size-fits-all" approach cannot work in the diverse contexts of global health. WHO has established a one-stop support mechanism, which offers a centralized source of tools, technical guidance, and best practices for countries to access.

At the heart of this support is the Operational Framework for PHC which is a practical guide that helps translate broad PHC principles into real-world policies and interventions. Additionally, this strategy incorporates lessons learned from the COVID-19 pandemic to guide build back better approach that directs investments and political support towards more resilient and people-centered health systems. By grounding its recommendations in the lived experiences of countries worldwide, WHO ensures this support remains both actionable and effective

2. Fostering evidence-based innovation with a sharp focus on equity

The second strategic area focuses on generating and applying evidencebased innovations, especially to reach populations often excluded or left behind by traditional health systems. Recognizing that sustainable progress requires continuous learning and adaptation, this pillar supports countries in developing data driven, equity focused solutions.

A major initiative under this pillar is the creation of monitoring and measurement tools, enabling countries to track progress and enhance accountability. These efforts will culminate in a Global Report on PHC Progress, providing a comprehensive overview of strengths, gaps, and opportunities globally.

Another key initiative is the WHO Academy's capacity-building program, which aims to train a new generation of health professionals and leaders with the skills to lead PHC reforms. The overarching goal is to ensure that health decisions are grounded in evidence and consistently prioritize equity.

3. Promoting policy leadership and strategic partnerships

The third pillar acknowledges that technical solutions alone are not enough without strong political will and public engagement. This pillar aims to strengthen PHC by

promoting leadership, advocacy, and partnerships across all levels from governments to community-based organizations.

WHO is actively working to build a broad network of support, including governments, NGOs, development agencies, UN bodies, youth leaders, parliamentarians, and other stakeholders. By incorporating diverse voices, especially those of younger and emerging leaders, the strategy aims to foster inclusive, community-driven health systems.

To further energize global commitment, WHO is establishing a Strategic Advisory Group on PHC and launching a PHC Award to recognize excellence and inspire innovation in PHC implementation globally

Primary health care in Ethiopia

In 1974 Ministry of Public Health, and the first National Health Service was established. But it was largely unsuccessful at first, for the following reasons:

- The policies and strategies for the specific elements of the PHC were not clearly defined at national level
- Regions and health facilities had limited awareness of those elements as defined at the central level.
- There was lack of clarity on health policies in most regions as a result of poor and inadequate dissemination of information on the policies

The Alma-Ata Declaration on Primary Health Care (1978), and its call for Health for All by the year 2000, was integrated in the in the health system soon after its declaration. This lead to the community based preventive and promoting health services.

In 2002 health extension program was launched which was a major milestone in the development of the PHC. Its aim was to deliver preventive, promotive and basic curative care to rural households. It is implmented through a trained female health extension workers deployed across the country. It has 16 packages grouped in to

- hygiene and environmental sanitation
- disease prevention and control
- Family health services
- health education and communication

Health extension program was originally rural based but after seeing the benefits of such services in 2009 urban extension program was launched with a 24 packages aiming to address communicable disease, poor sanitation and the lifestyle modification.

Currently, PHC is guided by the Health Sector Transformation Plan and the Woreda (district) Based Health Sector Plan, implemented through the Health Extension Program (HEP). Through this, PHC coverage has grown rapidly over the last 16 years. Ethiopia's health system is structured in to three-tier system: primary, secondary and tertiary level.

The primary level includes primary hospitals, health centers and heath posts.

Primary health care unit consists of one health care and five satellite healthposts and it provides primary health care services at the community level.

Primary hospitals-provide an inpatient and outpatient care; perform emergency surgeries, referral center for the health centers, and to train nurses and paramedics

Health centers – provide service about to 25,000 people and supervise five health posts. They provide outpatient, maternal child health and delivery services.

Health posts- are the lowest level of care locate at the kebele level and each serves around 5000 people. Two HEWs work at the each HP and they will report to the HC and the kebele administration.

Challenges for implementation of Primary health care

- shortage of well-qualified physicians and health-care workforces to serve the patients in emergency,
- inadequate technology and equipment
- poor condition of infrastructure, especially in rural areas,
- concentrated focus on curative health services rather than preventive and promotive health-care services,
- challenging geographic distribution,
- poor quality health-care services,
- inadequate financial support in health-care programs,
- lack of community participation,
- poor allotment of health-care workforce to serve in the rural locality, and
- lack of intersectoral collaboration.

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